INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Tepotinib monotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

**Treatment pattern**
**Ongoing**
Follow the prescribed daily plan

**Regimen source**
**NCCP 00823**
Version 2

Daily treatment with food

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

SACTCHECK · CHAPTER 01

# Understanding your treatment

Tepotinib monotherapy · National Cancer Control Programme (NCCP) 00823

## How this medicine supports your treatment

Tepotinib blocks the MET growth signal in selected lung cancers with a MET exon 14 skipping change. In advanced cancer the aim is to control the cancer and its symptoms. The tumour test and previous treatment help determine whether it is suitable.

MET TARGETED THERAPY

### Tepotinib

**Blocks abnormal MET growth signalling**

MET exon 14 change identified → MET signal blocked → Cancer growth slowed

Tepotinib inhibits the MET receptor. A MET exon 14 skipping change can keep this growth signal active; blocking it can slow or shrink selected lung cancers.

Report new cough or breathlessness, swelling, severe bowel symptoms, jaundice, palpitations or fainting promptly.

## Your treatment aim

### Which setting?

NCCP 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

NCCP 00823 v2 · official regimen · EMA Tepmetko product information, updated 08/10/2026

### Dose and duration

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

## Evidence for this treatment

VISION found that tepotinib could shrink some advanced MET exon 14 skipping lung cancers, including cancers previously treated with other medicines. It was a single-arm study, so it does not show how much longer people live compared with another treatment. Study results describe groups and cannot predict your own outcome.

Single-arm evidence does not establish comparative survival benefit; first-line trial patients are outside this NCCP indication.

## When to get help

**Emergency: 112 / 999.** Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

**Contact your treating team promptly:** Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit's temperature and 24-hour contact instructions.

YOUR ONCOLOGY TEAM · DAYTIME

**Insert numbers here**

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

**Insert numbers here**

Ask for medical oncology

EVENINGS / OVERNIGHT

**Your agreed out-of-hours pathway**

Other hospitals: use the contacts your own team provides.

SACTCHECK · CHAPTER 02

# Your schedule, tests and appointments

Tepotinib monotherapy · NCCP 00823

BASELINE → TREATMENT → FOLLOW-UP

## Checks alongside continuous tablets

Relative stages, not equal time periods or booked appointments. NCCP does not specify a universal number of weeks between these checks; your team agrees the schedule.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00823 v2 · official regimen · EMA Tepmetko product information, updated 08/10/2026

THE WHOLE TREATMENT JOURNEY

## Tepotinib monotherapy: treatment and checks

**Your treatment pattern**

Daily treatment with food

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

### Advanced MET exon 14 skipping non-small-cell lung cancer (NSCLC) · after prior systemic therapy

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before the first dose | Starting daily treatment | Ongoing reviews | Benefit or toxicity review | Change or stopping |
|---|---|---|---|---|---|
| **Treatment** NCCP 00823 · SMPC | Confirm prescription | Daily tepotinib with food | Continue authorised daily dose | Discuss benefit and side effects | Team authorises change or stopping |
| **Cancer and eligibility** NCCP 00823 · SMPC | MET test and previous treatment | Confirm prescription | Scans under the Consultant plan | Review response and symptoms | Agree the next plan |
| **Blood and organ function** NCCP 00823 · SMPC | Blood count, electrolytes, kidney and liver tests | Review results and early symptoms | Repeat regular tests | Extra tests for abnormalities | Follow unresolved effects |

| Care / check | Before the first dose | Starting daily treatment | Ongoing reviews | Benefit or toxicity review | Change or stopping |
|---|---|---|---|---|---|
| **Heart** NCCP 00823 · SMPC | electrocardiogram (ECG) when indicated | Check risk and interacting medicines | ECG/ electrolytes when indicated | Assess fainting or palpitations urgently | Confirm the follow-up plan |
| **Symptoms and practical support** NCCP 00823 · SMPC | Discuss lungs, swelling and bowel plan | Take with food; check supply | Review symptoms, weight and adherence | Authorised hold or dose decision | Continue contact advice and contraception |

## What each check is for

### 1 Confirm eligibility

Before the first dose

Treatment depends on a specific tumour alteration and previous treatment.

**Ask / act:** Document the NCCP 00823a indication and agreed prescription.

### 2 Keep tests and symptoms connected

At scheduled reviews and between visits

Blood tests can change before symptoms and symptoms can occur between appointments.

**Ask / act:** Confirm who orders tests and reviews results; give an urgent contact plan.

### 3 Possible interstitial lung disease (ILD) or liver injury

At any time

Suspected lung inflammation and significant liver abnormalities can require an immediate hold.

**Ask / act:** Urgent clinical assessment and an authorised treatment decision.

### 4 Benefit or toxicity changes

At treatment reviews

There is no fixed chemotherapy course length.

**Ask / act:** Agree continuation, an authorised hold/restart, or the next treatment plan.

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Before and during treatment

Tests and symptoms guide treatment decisions.

### Before starting

Confirm the MET result, previous treatment, dose, other medicines and emergency contact. Check blood count, electrolytes, kidney and liver function; ECG if clinically indicated. Discuss pregnancy, contraception and breastfeeding, and perform a pregnancy test when applicable.

### Ongoing reviews

Repeat blood count, electrolytes, kidney and liver tests at the team's agreed reviews. ECG is used when clinically indicated. Report new lung symptoms, swelling, diarrhoea or vomiting between visits. A rise in creatinine needs assessment because the medicine can change the test result without necessarily damaging the kidneys. Scans follow the Consultant's plan.

## Taking tepotinib

Follow your own prescription.

### Food and missed doses

Take at the same time each day with food and swallow whole. If a dose is missed, take it when remembered on the same day unless the next dose is due within 8 hours; then skip it. Do not double a dose. Ask your pharmacist if the timing is uncertain or if you vomit; do not take a replacement without advice. If swallowing is difficult, ask about the approved water-dispersion method.



### Medicine interactions

Ask the pharmacist to check all prescribed medicines, non-prescription medicines and supplements before changes. Tepotinib can affect medicines including dabigatran, digoxin, rosuvastatin and metformin. Supportive medicines may also affect heart rhythm; individual advice is needed.

EMA Tepmetko product information, updated 08/10/2026

## Practical treatment details

### Supplies and reviews

Arrange repeat prescriptions before tablets run out.

The tablet regimen itself needs no infusion line or pump.

# Symptoms and supportive care

Tepotinib monotherapy · NCCP 00823

## Daily tablets, tests and treatment decisions

No dates or personal information are needed. Confirm your actual appointments with the team.

| Treatment or check | Timing in this view |
|---|---|
| **Preparation** | **Before the first dose**<br>Confirm the MET result, prior treatment, prescription, blood tests and medicines. |
| **Daily oral treatment** | **Starting daily treatment → Ongoing reviews → Benefit or toxicity review**<br>The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects. |
| **Tests and symptoms** | **Starting daily treatment → Ongoing reviews → Benefit or toxicity review → Change or stopping**<br>Regular blood tests, symptom review and electrocardiogram (ECG) when indicated; report changes between visits. |
| **Next treatment decision** | **Change or stopping**<br>Continue only while benefit is observed and the team considers the treatment appropriate. |

Take at the same time each day with food and swallow whole. If a dose is missed, take it when remembered on the same day unless the next dose is due within 8 hours; then skip it. Do not double a dose. Ask your pharmacist if the timing is uncertain or if you vomit; do not take a replacement without advice. If swallowing is difficult, ask about the approved water-dispersion method.

Columns show sequence, not equal periods. A hold does not authorise you to restart from this timeline.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

## Support during treatment

Report changes early so your team can help.

### Swelling and weight changes

Report swelling early and discuss changes in weight, movement or breathing. The team will assess the cause and agree treatment; do not start a water tablet or change tepotinib yourself.

EMA Tepmetko product information, updated 08/10/2026

### Diarrhoea and fluids

Agree a diarrhoea treatment plan before starting. Begin the prescribed anti-diarrhoeal medicine at the first loose stool as instructed, and maintain fluids if able. Persistent diarrhoea, weakness or reduced urine needs prompt advice.

### Nausea medicines

Tepotinib has minimal-to-low oral emetic risk. Agree an as-needed anti-sickness plan with the team, including medicine-interaction and heart-rhythm checks; no routine infusion premedication is specified.

NCCP 00823 v2 · official regimen · NCCP systemic anticancer therapy (SACT) Antiemetic Guidance V6 (2025), oral therapy Table 5

### Medicine review

EMA Tepmetko product information, updated 08/10/2026

### Pregnancy, fertility and breastfeeding

Avoid pregnancy during treatment. Effective contraception is needed during treatment and for at least one week after the last dose; women using hormonal contraception should also use a barrier method. Men with a partner who could become pregnant should use barrier contraception for the same period. Do not breastfeed during treatment or for at least one week afterwards. Human fertility effects are not known; discuss fertility before starting.

## When to get urgent help

Do not wait for the next appointment.

### Call urgently

Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

### When you call

Give the medicine name, last dose and symptoms. Do not delay calling while gathering information.

## What studies tell us

### Your own decision

Ask how closely the trial population matches your situation and discuss alternatives and remaining uncertainties.

VISION long-term follow-up, Mazieres et al., JAMA Oncology 2023 · EMA Tepmetko product information, updated 08/10/2026

## When treatment changes

The team authorises continuation, a pause, restart or stopping.

### Review benefit and side effects

Agree who will review test results and confirm the dose. A source reduction to 225 mg is a clinician decision, not an instruction to halve tablets yourself.

### Follow-up

Confirm the next treatment or follow-up plan, symptom advice and precautions after the final dose.

## Practical support

### Swelling and weight changes

### Diarrhoea and fluids

### Nausea medicines

### Medicine review

### Pregnancy, fertility and breastfeeding

Taking tablets / attending treatment

## Questions to ask

**Is this a fixed course of chemotherapy?**

No. These targeted tablets are taken daily without a planned rest week, while they are helping and your team considers treatment appropriate.

**Why does a trial mention 500 mg?**

The VISION trial's 500 mg salt dose corresponds to 450 mg active tepotinib. The prescribed tablets contain 225 mg active tepotinib each; follow your own prescription.

**What if I forget or vomit a dose?**

Take a forgotten dose when remembered on the same day unless the next dose is due within 8 hours; then skip it. Never double up. Ask your pharmacist about vomiting or uncertain timing before taking any replacement.

**Why might my creatinine test change?**

Tepotinib can change how creatinine is handled by the kidneys, but real kidney problems can also occur. The team must assess the change; neither explanation should be assumed.

**Can I change the dose when I feel better?**

Only your team can authorise restarting or changing the dose after side effects. Feeling better does not establish that blood tests or other checks have recovered.

**Explain the plan back:** Can you explain why the MET result and previous treatment matter, how you take the tablets with food, what to do if a dose is missed, and which symptoms mean you should call today?

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**Advanced MET exon 14 skipping lung cancer** NCCP 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Tepotinib alone · daily treatment

**Active medicines:** tepotinib

**Emetogenic risk:** Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

### Additional phase-specific support

- Report swelling early and discuss changes in weight, movement or breathing. The team will assess the cause and agree treatment; do not start a water tablet or change tepotinib yourself.
- Agree a diarrhoea treatment plan before starting. Begin the prescribed anti-diarrhoeal medicine at the first loose stool as instructed, and maintain fluids if able. Persistent diarrhoea, weakness or reduced urine needs prompt advice.
- Tepotinib has minimal-to-low oral emetic risk. Agree an as-needed anti-sickness plan with the team, including medicine-interaction and heart-rhythm checks; no routine infusion premedication is specified.
- Ask the pharmacist to check all prescribed medicines, non-prescription medicines and supplements before changes. Tepotinib can affect medicines including dabigatran, digoxin, rosuvastatin and metformin. Supportive medicines may also affect heart rhythm; individual advice is needed.
- Avoid pregnancy during treatment. Effective contraception is needed during treatment and for at least one week after the last dose; women using hormonal contraception should also use a barrier method. Men with a partner who could become pregnant should use barrier contraception for the same period. Do not breastfeed during treatment or for at least one week afterwards. Human fertility effects are not known; discuss fertility before starting.
- The written diarrhoea prescription and escalation instructions should be confirmed before treatment. NCCP provides loperamide dosing or local-policy options; these must be individualised.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

**Advanced MET exon 14 skipping lung cancer** NCCP 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

NCCP 00823 · VERSION 2

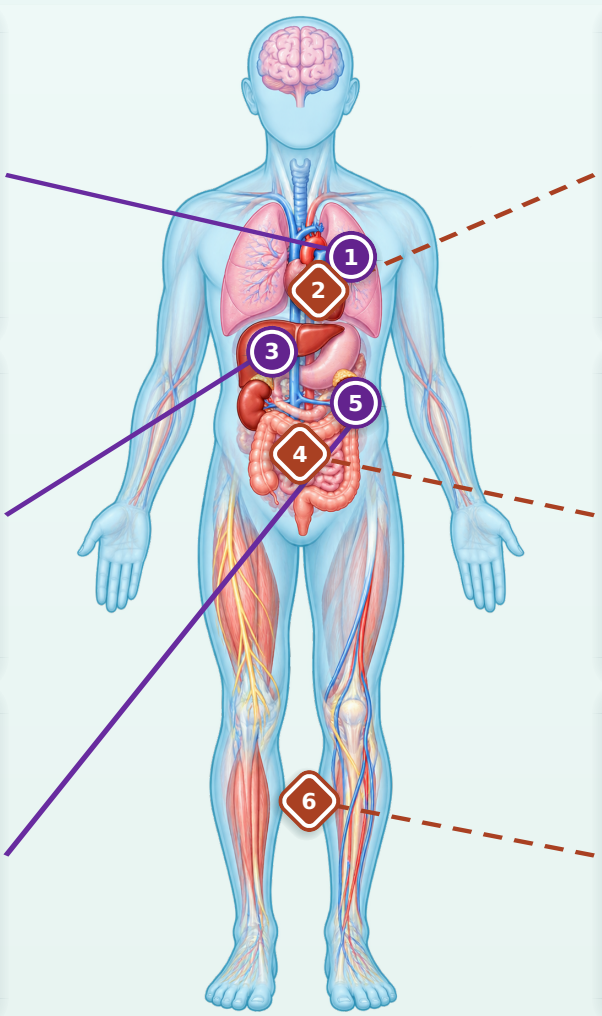
# Tepotinib monotherapy

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

## A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.




### 1 New cough or breathlessness

**tepotinib**

Possible lung inflammation needs urgent assessment.

### 2 Palpitations or fainting

**tepotinib**

Possible heart-rhythm changes need assessment.

### 3 Yellow skin or eyes

**tepotinib**

Liver changes may be found in tests or cause symptoms.

### 4 Diarrhoea or vomiting

**tepotinib**

Early support helps prevent dehydration.

### 5 Kidney-test changes

**tepotinib**

Creatinine changes need assessment, including causes other than kidney injury.

### 6 Swelling and weight gain

**tepotinib**

Fluid retention can affect movement or breathing; report changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

SACTCHECK · CHAPTER 04

# Your record and contact card

Tepotinib monotherapy · National Cancer Control Programme (NCCP) 00823

Bring this to reviews. Record symptoms, treatment changes and questions when helpful. You do not need to complete a daily diary or calculate a toxicity grade.

The four sheets are reusable review periods, not a prescribed number of cycles. Follow the treatment and rest days agreed with your team; do not calculate a restart from this sheet.

These patient-language prompts are not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) assessment. Urgent symptoms override the form.

Review period / visit: _____ Next review: _____

| Medicine-specific symptom | No concern | Present / manageable | Affects sleep or daily activities — contact team | When / what changed? |
|---|---|---|---|---|
| **Swelling / sudden weight change** | ☐ | ☐ | ☐ | |
| **New cough or breathlessness** | ☐ | ☐ | ☐ | |
| **Diarrhoea** | ☐ | ☐ | ☐ | |
| **Nausea** | ☐ | ☐ | ☐ | |
| **Yellow skin or eyes** | ☐ | ☐ | ☐ | |
| **Palpitations or fainting** | ☐ | ☐ | ☐ | |

## Checks: expected, booked and reviewed are different

| Test / review | Due window | Who books it? | Who reviews the result? | Outcome / next action |
|---|---|---|---|---|
| _____ | _____ | _____ | _____ | _____ |
| _____ | _____ | _____ | _____ | _____ |
| _____ | _____ | _____ | _____ | _____ |

Missed tablets / injection changes / questions / support needed:

Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit's temperature and 24-hour contact instructions. Emergency symptoms: 112 / 999.

# Carry your treatment details

**Advanced MET exon 14 skipping lung cancer** NCCP 00823a covers tepotinib on its own for adults with advanced non-small cell lung cancer carrying a confirmed MET exon 14 skipping change, after immunotherapy and/or platinum chemotherapy. It is not a first-line or adjuvant treatment guide.

Print, cut out and fold. Follow your treating hospital’s contact plan.

SACTCHECK · TREATMENT CARD

## Tepotinib monotherapy

NCCP 00823 v2

The usual starting dose is 450 mg once daily: two 225 mg tablets taken together with food. Treatment is continuous, without a planned rest week, while it is helping. Follow your prescribed dose; the team may use one 225 mg tablet or pause treatment for side effects.

Prescriber / next review: _____

**Contact plan**

Oncology team: Insert numbers here · Use your hospital’s opening hours.
Out of hours: Insert numbers here.
Evenings / overnight: agreed out-of-hours service or emergency department.
**Emergency: 112 / 999.**

Own hospital / contact: _____

Contact the oncology team urgently for new or worsening cough, breathlessness or fever; marked swelling or sudden weight gain; severe diarrhoea, repeated vomiting or inability to drink; yellow skin or eyes, dark urine or severe abdominal pain; or palpitations or fainting. Severe breathing difficulty, collapse or swelling of the mouth or throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions.

https://sactcheck.com/patient/00823/ · no patient information encoded.

SACTCheck · Developed by Dr Paul O’Brien · v0.84.25 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending

# Sources and further information

Tepotinib monotherapy · National Cancer Control Programme (NCCP) 00823

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **NCCP 00823 v2 · official regimen**
   https://healthservice.hse.ie/documents/6868/823_V2_Tepotinib_Therapy.pdf
2. **EMA Tepmetko product information, updated 08/10/2026**
   https://www.ema.europa.eu/en/documents/product-information/tepmetko-epar-product-information_en.pdf
3. **VISION long-term follow-up, Mazieres et al., JAMA Oncology 2023**
   https://jamanetwork.com/journals/jamaoncology/fullarticle/2805800
4. **VISION treatment sequencing: ≥3-year follow-up, ESMO 2025 abstract 1995P**
   https://www.annalsofoncology.org/article/S0923-7534(25)03539-2/fulltext
5. **VISION sustained-response subset, ASCO 2026 abstract 8612**
   https://ascopubs.org/doi/10.1200/JCO.2026.44.16_suppl.8612
6. **NCCP V6 Table 5: as-needed antiemetics for minimal-low oral therapy; NCIS V8 parenteral schedules do not impose a routine oral prescription**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
7. **NCCP NCIS supportive antiemetics V8: parenteral guidance**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf